Chapter 6

Communication Skills and Patient History Interview

Shaun Wen Huey Lee[1], Dixon Thomas[2], Seeba Zachariah[2] and Jason C. Cooper[3]
[1]*Monash University Malaysia, Bandar Sunway, Malaysia;* [2]*Gulf Medical University, Ajman, United Arab Emirates;*
[3]*Medical University of South Carolina, Charleston, SC, United States*

Learning Objectives

Objective 6.1 Explain why communication skills are essential for pharmacists.
Objective 6.2 Detail the elements of professional communication.
Objective 6.3 Identify barriers to communication and suggest how to overcome them.
Objective 6.4 Discuss the characteristics and process of the patient history interview.

OBJECTIVE 6.1. EXPLAIN WHY COMMUNICATION SKILLS ARE ESSENTIAL FOR PHARMACISTS

Good communication plays an important role in patient-centered care and collaborative practice. While considering communication as a skill in both clinical practice and life, it takes time and effort before one becomes comfortable and proficient. As such, one should take time to reflect after an interaction about what went well, what did not, and what contributed to the success/failure of the communication. It can help the pharmacist to identify further aspects that can be strengthened, as well as being an extremely powerful developmental exercise. Ideally, the development of communication skills should happen in a comfortable setting, such as with family/friends or in educational settings. Pharmacists who do not possess sufficient communication skills will find it difficult to work effectively.[1–5]

Pharmacists should have good reading, listening, speaking, and written communication skills. The order given here is intentional. Traditionally, pharmacists are known for their skills to read prescriptions and interpret their meaning into action. Reading and deciphering medical records requires the application of pathophysiology and pharmacotherapy knowledge. Reading skills are also essential in evidence-based practice to read and interpret clinical literature. Listening involves hearing from patients, other consumers, caregivers, healthcare professionals, and administrators. In a typical counseling session, the pharmacist listens for more time than he/she speaks. Collecting appropriate information happens only when the pharmacist listens to and observes all signals from medical records, patient explanations, signs/symptoms, and patient behavior. Speaking is a way of giving advice. Such guidance should be trustworthy, assertive enough to change wrong practice, and empathic enough to be accepted. Written skills are especially important when providing drug information, writing in medical records, and other clinical communications. The combination of these important communication skills will result in better patient adherence and outcomes, prevention of medication errors, and an efficient healthcare system.

The scope of pharmacy practice now also includes patient-centered processes such as providing drug information and direct patient counseling, as well as technical aspects of pharmaceutical care such as medication reconciliation. Collection of information from patients as part of pharmaceutical care or the newer term pharmacists' patient care process (PPCP) requires improved communication skills. As such, it is essential that pharmacists become an effective healthcare team member and possess the skills and attitudes to enable them to assume their roles appropriately. Indeed, this has been highlighted by several organizations including the Joint Commission International, which identified in their patient safety

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goals for the need to "improve effective communication."[6] This has been similarly endorsed by the World Health Organization, in which a pharmacist has been identified as the communicator or the link between patient and other healthcare professionals.[7] As per the Center for the Advancement of Pharmacy Education (CAPE) communicator is an educational outcome.[8] Pharmacy programs include communication as a core element of their curriculum.

Pharmacists' Communication Skills Relevant to Consumers

Pharmacists regularly communicate with consumers and healthcare professionals. Application of relevant communication skills helps consumers to understand what they should know. Understanding the audience or whom we are interacting with about their likes and dislikes, and information needs is a foundation in professional communication. For example, a patient may swallow a suppository if the route of administration is not communicated well. Studies show that communication gaps result in medication errors; subsequently, the healthcare system fails to produce optimal outcomes.[2] Strong communication skills improve pharmacist–consumer interactions by

- Being a highly accessible healthcare professional.
- Establishing the necessary rapport to build a trusting relationship.
- Being a trusted consultant in the use of over-the-counter (OTC) as well as complementary alternative medicine products.
- Persuading consumers to adhere to their medication.
- Communicating essential information that guides consumers on the safe use of their medications.
- Tailoring and directing the communication process as the situation demands.
- Providing advice on other healthcare needs, such as preventive health (e.g., vaccinations, health screenings, physicals).

Pharmacists' Communication Skills Relevant to Other Healthcare Professionals

Communicating to healthcare professionals are in professional language expressing pathophysiological and pharmacotherapy knowledge in technical terms for exchanging deep scientific information. Clinicians appreciate evidence-based information to develop or modify patient care decisions while patients appreciate if pharmacists communicate same concepts in lay language. Developing and implementing pharmaceutical care plans requires efficient teamwork. Teamwork based on effective communication is the cornerstone of today's collaborative practice. Interprofessional communication is promoted in healthcare education, practice, and research.[9] Phases of team performance described by Psychologist, Bruce Tuckman, is well known. According to him, a group goes through the following phases to perform[10]:

- Forming
- Storming
- Norming
- Performing

Rightly said, when a healthcare professional team is under development, the communication gaps between doctors, nurses, and pharmacists are experienced by all. In the storming stage, criticism may arise regarding what is going wrong. Later, when rapport is established, professionals understand the shared responsibility and work in harmony (i.e., normalization). As a result, patients greatly benefit from the enhanced performance of multiprofessional healthcare teams as efficient teamwork is found to reduce medication errors and other drug-related problems.[2,11,12]

OBJECTIVE 6.2. DETAIL THE ELEMENTS OF PROFESSIONAL COMMUNICATION

Interpersonal communication is a fundamental aspect of clinical practice. It is a two-way process where both the patient and pharmacist can give information through verbal and nonverbal messages. The interpersonal communication model comprises five key elements (Fig. 6.1):

- Sender—person conveying the message
- Message—information conveyed (verbal and nonverbal)
- Receiver—the person receiving the message
- Feedback—receiver communicates their understanding of the sender's message
- Barrier—any interference with the expression or understanding of the message

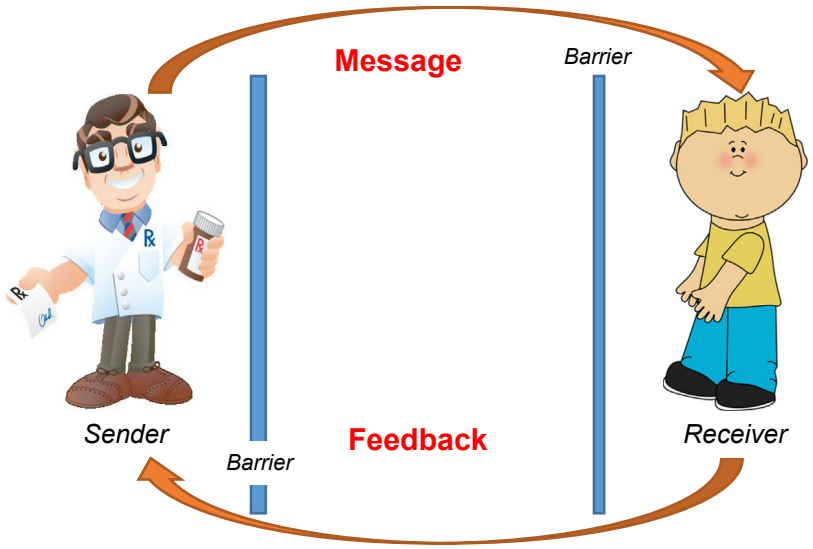


FIGURE 6.1 Interpersonal communication model. *Adapted from Beardsley RS, Kimberlin CL, Tindall WN.* Communication Skills in Pharmacy Practice: A Practical Guide for Students and Practitioners. *Lippincott Williams & Wilkins; 2012.*

Key Communication Skills

Research suggests that a message is communicated via three methods, namely words, the tone of voice, and body language. The factors facilitating the communication or its barriers are not always physical, it might be behavioral or situational too. Details of these key components are described further (Fig. 6.2).[14]

Body Language or Nonverbal Language

Body gestures, facial expressions, and eyes can speak a thousand words. As such, being able to interpret body language helps us to know how a patient feels during their consultation and the extent to which they are comfortable during the conversation.

Body language is the transmission and interpretation of one's feeling, attitudes, and moods via the following:

- Body posture, movement, position, and relationship to other objects and surroundings
- Facial expression and eye movement

As such, it is important to understand a patient's body language, as this will help identify points in the consultation where the patient may feel uncomfortable, confused, or disagree with something said. For example, a patient may have

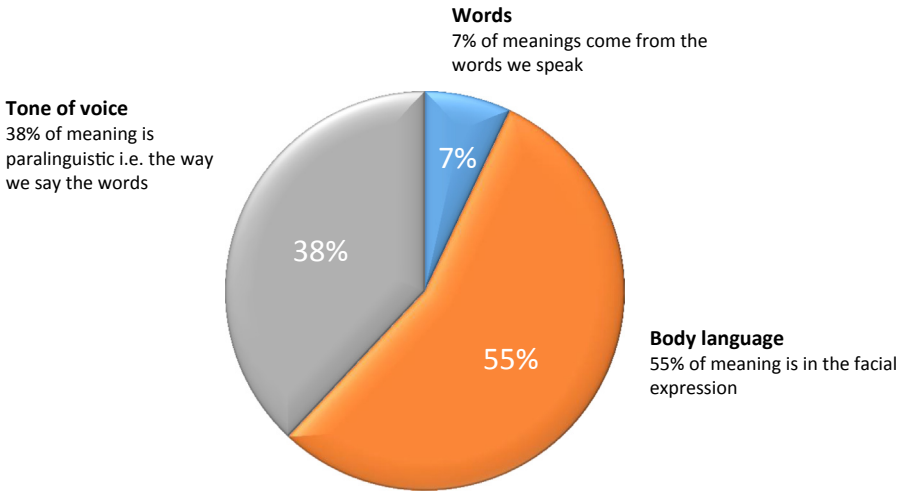


FIGURE 6.2 Components on how messages are communicated. *Adapted from Mehrabian A.* Silent Messages: Implicit Communication of Emotions and Attitudes. *2nd ed. Belmont, California: Wadsworth Publishing Company; 1981.*

certain beliefs about their medication (e.g., that the medication may result in side effects, or that it is not "natural", many are scared of chemical or genetically engineered products), which may affect their adherence. They will usually not offer this information, as they may feel their opinions will be brushed aside or misunderstood. Subsequently, it is important for a pharmacist to be able to identify some of the nonverbal cues associated with a patient's beliefs, including crossing their arms or hands during a consultation, showing a reluctance to listen, or trying to rush off during the consultation.

Verbal Language

Language is important in any consultation, either in the choice of words used or how information is being conveyed. One general rule of thumb is to avoid medical jargon and terminology, as this helps give the assurance that messages are communicated effectively. However, there is also a strong need to reflect the language and manner in which the patient speaks. For example, if the patient uses medical words during the consultation, choosing to respond in layman's terms may send out the wrong signals, as either they are not being listened to or the patient's knowledge is not being respected. As such, language and the way a patient uses language to communicate in a context is important; ultimately, this will help build rapport between the pharmacist and the patient.

In our increasingly diverse communities, it is also important to consider whether the pharmacist knows the language spoken by the patient. If not, steps should be taken to ensure that essential information can be provided by and to the patient either through a family member, another member of the pharmacy staff who speaks the patient's language, or an interpreter, if available.

Tone of Voice

The tone of voice, inflection, or nuance can help contextualize a message. An example is saying "no" in a firm tone when disagreeing with something, suggesting that the patient is adamant that this should not be done. The tone of voice will also produce different effects although the identical words may be spoken. For example, a sarcastic or threatening tone will produce different effects/emotions when compared with an empathetic tone.

Reflective Exercise

Have you ever considered assessing your own communication styles (e.g., body language, the tone of voice, and verbal language)? Try asking a family member or friend to take a video of yourself when you are in conversation with others. Watch this video and make notes on the language you portray. What changes will you make so that your communication is more effective?

Active Listening

One of the key components of effective communication is active listening. Listening is an approach to know the audience. Any communication will not be as effective without understanding the person, situaton and needs to be addressed through communication. It involves not only using the ears but also consists of a conscious effort to pay complete attention to the facial expression, body language, and verbal tone of the patient. It also involves active participation by the pharmacist (i.e., it is necessary to respond to the patient in a manner that demonstrates that the patient has both been heard and understood). In this manner, any responses can also serve to clarify the accuracy of understanding. In general, there are several types of responses which can be given.

- Paraphrasing: A response that repeats the words heard, as well as some superficial recognition of the patient's attitude or feeling. This is best used in the initial stages of patient interaction by restating phrases to reassure the patient that they are being listened to and to encourage them to continue communicating. For example, to check for the accuracy of their statement, reword the information provided in the form of a question back to the patient (i.e., "Are you saying … ?").
- Summarizing: A response that concisely reiterates the main points of interaction or consultation. Highlight any key critical points and allow the patient to add any new information that they may have forgotten. This type of response is most important to identify any misunderstandings that may exist, especially if there are barriers to communication (e.g., language barriers). Take this opportunity to form an agreement with the patient regarding the information discussed.

- Reflection: A response that verbalizes both the content presented and the feelings of the patient. It has the advantage of showing the patient that the pharmacist is paying attention to verbalizing words/information, as well as the emotions behind them. Usually, responses often begin with phrases such as "It sounds like you are experiencing …" or "You seem to be feeling ….". By communicating back to the patient that their feelings or concerns have been understood and are valid, a caring trusting relationship can be established.
- Clarifying: A response which questions or restates the content and feeling of the information presented. It can also be used to summarize the patient's statements into a clear, concise account. This response can begin with a phrase such as "As I understand it, you …". These statements allow for the patient to correct or reframe their understanding, if necessary.

Empathy

Empathy is the process of communicating with patients that the pharmacist understands the patient's perspective about their disease,[15] medications, and overall health. In simpler terms, this is considered "putting one's self in the patient's shoes." It is a core ingredient in any healthcare provider–patient relationship.[16] Empathy can be learned and requires that the pharmacist place importance on developing a caring response with the patient. This term should not be confused with sympathy. Sympathizing is feeling sorry for but many patients do not like showing a feeling of pity. Empathy is a more sharing approach feel the difficulty together with respect to autonomy. In empathy, the pharmacist needs to demonstrate how they respond to patients, both verbally and nonverbally. It does not necessarily require the pharmacist to experience the particular incident but will require the pharmacist to be open and able to acknowledge the feelings of the patient.

Demonstrating empathy through verbal responses means choosing words that do not judge, give advice, quiz, or placate. Rather, the words chosen should demonstrate understanding and acceptance of the patient. It should be acknowledged appropriately. For example, "I can see this is difficult for you to discuss …" or "It must be difficult for you to manage these new medicines with no one at home to help you …."

Reflective Exercise

Empathy is showing a response that demonstrates you share or acknowledge a person's feelings. Think about an occasion when someone you cared for shared some good news. What was your reaction as you shared their joy? Did any words convey your excitement, encouragement, or congratulations?

OBJECTIVE 6.3. IDENTIFY BARRIERS TO COMMUNICATION AND SUGGEST HOW TO OVERCOME THEM

In many situations, there are times when a consultation does not go as expected. These could be due to several reasons:

- Lack of skills and understanding of the structures of discussion and conversation.
- Inadequate knowledge of other forms of communication skills, such as body language.
- Lack of personal insight into other people's communication difficulties.
- Personal barriers, including stress, lethargy, shyness, and lack of time, to engage in meaningful discussion.
- Language barriers, including difficulty in speaking the language that patient understands, or cultural differences.
- Physical barriers, including lack of counseling rooms, cluttered desk, and noisy environments.
- Personal barriers (patient), including anxiety about their health or anger toward a previous incident (e.g., error in dispensing).
- Lack of knowledge by the pharmacist regarding medications prescribed or patient history.

As such, identifying these barriers are key steps to overcoming them. Once these barriers are identified, a pharmacist can then create and implement a solution. Some suggestions that could be used to reduce these communication barriers include the following:

- Reduce clutter and the number of products on sale near the counseling area.
- Use support staff, such as technicians and assistants, for other roles to effectively free-up time to speak with patients.
- Supplement patient counseling with print materials to increase understanding of information.
- Maintain eye contact with the patient. It will help the pharmacist to watch for any nonverbal cues that indicate a lack of understanding or concern.

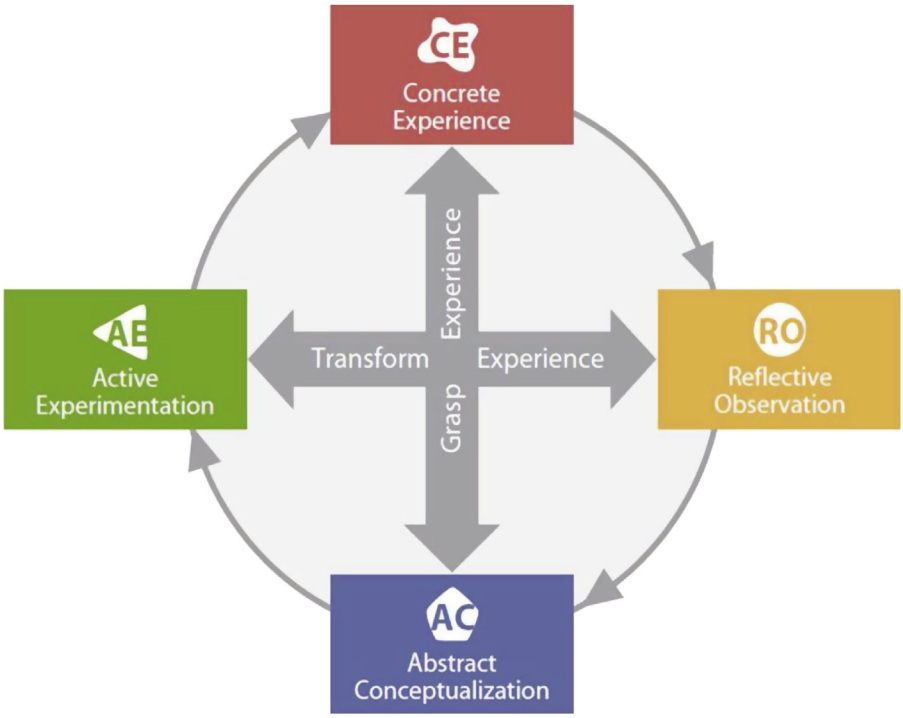


FIGURE 6.3 The experimental learning style. *Courtesy of Experience Based Learning Systems, Inc. Alice & David Kolb.* The Experiential Educator: Principles and Practices of Experiential Learning. *Kaunakakai, HI: EBLS Press; 2017.*

- Encourage patients to ask questions and thoughtfully respond to them.
- Wear clothing that is reflective of the healthcare status, including the use of name tags.
- Develop an effective patient interview style to ensure the ability to gather all the information before providing patient care.

Knowing Kolb's learning styles will help categorize the patient learner type in tailoring the approach by the pharmacist. In 1984, Kolb described different learning styles as to how an individual learns (Fig. 6.3).[17] An understanding of how individuals learn can help pharmacists be more effective in their communication skills and also how they can best approach the patient, especially concerning patient education. As there is a tendency for everyone to teach using their style, it is helpful to know what styles can be consciously incorporated when educating patients about their medications and health. Briefly, this can be categorized into four different learning styles:

Divergers
- Prefer doing and experiencing
- Focus on generating ideas and solutions
- Value harmony, listen with an open mind

Assimilators
- Prefer observation and reflection
- Focus on ideas and concepts over individual needs; requires time to reflect before responding to questions or situations
- Value logic and has strong organizational skills

Convergers
- Prefer to begin with understanding reasons and concepts
- Prefer to focus on finding practical solutions; can be unemotional
- Value quick decision-making and leadership

Accommodators
- Prefer to dive in and trying things out
- Focus on efficient time and energy; enjoy finding useful, convenient solutions
- Value a direct approach when dealing with people and situations

Reflective Exercise

Which learning style most closely applies to you? Think how you would tailor the way you would teach others about their medications or health conditions when their learning preference may be different from your own?

Always demonstrate an open body stance. Some additional tips to ensure success include the following:

- Maintain a confident but empathic behavior that gives warmth to the patient and colleagues, versus an assertive behavior.
- Make sure the arms and legs are uncrossed, as folded arms may look defensive or imply decreased interest.
- Lean forward slightly when providing key facts to demonstrate active listening.

Many factors contribute to miscommunication in healthcare. Identifying these barriers and solving them are key to preventing harm.

OBJECTIVE 6.4. DISCUSS THE CHARACTERISTICS AND PROCESS OF THE PATIENT HISTORY INTERVIEW

Collecting subjective and objective information from the patient is one of the first steps in pharmaceutical care or the newer PPCP model. The first step is to collect. It is not easy to collect subjective information from a noncooperating patient. Especially with limited objective evidence, collecting accurate history uses effective communication skills, influence, diagnosis, and prognosis.

A patient history should be a comfortable discussion for the patient, but it should be skillfully directed by the pharmacist to collect important information. The patient should not feel like they are being interrogated. In addition, any admonishment of the patient from doing something wrong and suggesting that they may end up with a disease will likely result in less cooperation from the patient (e.g., diabetes from poor dietary habits). The interview session should be warm and encouraging to reveal helpful information about the patient. It should be as short as possible, except in some cases when the diagnosis or prognosis is significantly based on history interview.

Pharmacist–patient interactions involve usually no history taking while dispensing medication. History taking is performed when required. Some may need to conduct a detailed, structured interview, whereas in most other cases, a few short questions will do. Taking a relevant medical and medication history helps in deciding on the next appropriate steps, such as further assessment or referral to another healthcare professional.[18]

Involve the Patient in the Care Process

One of the key features in taking any patient history is the involvement of the patient as a stakeholder in clinical practice decisions. The patient history interview is a chance for the patient to discuss his/her health problems with a trusted partner in healthcare. It can also provide significant emotional support to the patient. A proper patient history interview results in a common understanding of the health condition of the patient. Early engagement of patients in contributing to clinical decisions may motivate them to be more of an active participant in their own self-care. An informed patient has higher chances of being adherent to therapy. The interviewer should not be judgmental, as expressions that communicate wrongdoing or disagreement in their overall health may demotivate patients to stay involved. Even when some items that patients say may be irrelevant to the current situation, the conversation should eventually be diverted back to what is significant.[18,19]

History Interview for Clinical Diagnosis

To tailor individualized therapy, perform a detailed patient history interview. In the complexity of atypical presentations of illness, patient assessment is highly supported with a proper interview. The patient history interview is important to rule out certain suspected conditions in the differential diagnosis. Many times, with accurate patient history interviews, the proper diagnosis can be made without inconvenient lab tests for the patients. The growth of newer technological support regarding diagnosis, however, has not decreased the value of the patient history interview. Many times, a combination of the patient history interview, physical assessment, diagnostic tests, and other technical support is required to arrive at a correct diagnosis.

The Patient History Interview Should Be Ethical

Cultural dimensions of the patients should be observed during the interview. Being interviewed by the person of opposite gender can also be a barrier for patients to open up completely. Such inhibition is extremely high in certain cultures, and collecting information about religiously or culturally restricted activities can be tricky. Asking questions about sexual practices or alcohol consumption is usually not well received by patients in some cultures but are sometimes important questions to ask. Sensitive issues should be handled carefully only if the interviewer feels they have the competence to manage certain situations; otherwise, such questions may need to be omitted for the time being, or another person of the same gender or a highly competent interviewer could be arranged to collect such information.

It is not just the information that is sensitive, but also, some patients may be sensitive themselves, especially following trauma or a psychiatric event. A competent psychologist may be needed to interview such patients without causing further psychological harm.[20]

The Information Collected Should Be Reliable

Patients should be encouraged to detail what they think is true about their conditions. Still, such information may be incorrect, as they are not experts in perceiving disease symptoms or making diagnoses. Many patients may intentionally lie to please the interviewer; they may underreport things that are bad for their health (e.g., smoking tobacco) and overreport things that are good for health (e.g., fruits, vegetables, exercise). If the patient does not like the way they are being interviewed, they may try to falsify or omit information as a part of being resistant. The interviewer should have appropriate communication skills to detect and prevent such intentional or unintentional wrong data being given. Clinicians can verify information collected from the interview with lab tests (e.g., diet control with blood glucose/hemoglobin A1C levels in diabetics). As one strategy alone is insufficient to diagnose and prognose the conditions accurately, a combination of subjective (e.g., observation, history interview) and objective (e.g., lab tests, scans, physical examination) methods may be adopted.

Some patient history interviews require the help of a third person:

- Parents to provide information about a pediatric patient.
- Interpreters to collect information about a patient who speaks a different language.
- Caregivers to collect information about a patient with sensory impairments.
- Caregivers or aid workers to collect information about patients with impaired cognition (e.g., psychiatric and neurologically disabled).[20]

Steps in the Patient History Interview

Many formats of a structured history interview are used by different healthcare providers. For example, the Calgary Cambridge model provides a good guide to the medical interview and communication process. It explains a systematic method of obtaining a patient history interview.[21] A summary of the common steps used in most patient history interviews are given below:

1. Collect appropriate background information
   Read about the patient if any information is available, as some patients might have already undergone a detailed history interview in the past. It is unnecessary to repeat questions on historical information unless further clarification is needed. Remember that the history interview is not a counseling session. Its main aim is to collect all the relevant information from the patient. Therefore, the patient should speak more than the interviewer.
2. Plan for the interview
   When a patient makes an appointment, it is important to tell them to bring a written list of problems they are facing, as well as the medicines (prescription/OTC) and herbal products they are taking. It will greatly help in identifying the patient's current medicines, because many patients do not remember the names of all the drugs they are taking. Such early engagement of the patient is likely to make them more actively involved in the care process. Depending on the information available, make plans as to how to start, conduct, and end the interview. Changes might be required to the original plan depending on how the interview goes. However, having an initial plan will greatly influence the outcome of the interview, as well as improve the skills of interviewer over time.
3. Try to know the patient
   Getting to know the patient is important in providing individualized care. The patient's behavior might not be the same since the last visit. As such, always take a moment to focus on the patient and task at hand. Always greet them by their name and remind them who you are. If possible, try to take a few moments to screen through and read any previous

encounters with the patient (e.g., dispensing records). Ask the patient to describe their problem, as this gives them the opportunity and time to talk about their situation. This can also help uncover problems that the patient has not acknowledged themselves, such as depression or anxiety. It also helps to develop a good relationship between the pharmacist and patient and promotes empathy and understanding. Understanding the cooperation level is important. Patients should be prepared to receive questions and be ready to answer.

4. Gather information
   This is the step mainly to gather and record the information. If some of the information has already been stated, do not repeat such questions. Gather all required information such as the following:
   - **a.** Social and family history: Some conditions have a familial relationship, whereas ethnic background may be another predisposing factor. Some conditions also may be epidemic in the society (e.g., influenza).
   - **b.** Vaccination and childhood illness history: This information can rule out risks for certain vaccine-preventable diseases, but the success rate of vaccines in preventing such diseases should also be considered. If some viral infections happened in childhood, the likelihood of recurrence is less. The reverse is also possible, as some childhood illnesses could have lingering effects. For example, children who are infected with the respiratory virus when they were young have a higher risk of wheezing and other signs of respiratory distress at a younger age.[22]
   - **c.** Medical and surgical history: It is important to obtain a history of diseases, both cured and ongoing. Medical procedures and surgeries undertaken shall also need to be documented.
   - **d.** Medication and diet history: List key medicines the patient has previously taken and those that are currently being taken, including prescription, OTC, supplements, and herbal products. Note adverse drug reactions to medications, as well as the time of occurrence, management, and outcome. Ask about drug and food allergies and food habits, which may previously suggest disease-specific diets.
   - **e.** History of present illness: Detail symptoms of the current illness that are bothering the patient. Obtain information about the occurrence and nature of the illness (e.g., how often does it occur, does it reoccur at a specific time).
   - **f.** Patient/social lifestyle: Ask about daily habits of the patient, including relevant hobbies, physical activity, alcohol, tobacco, and recreational drug use. Breach the subject of sexual activity, if necessary.
   - **g.** Adherence history: Document levels of patient adherence to previously prescribed therapy.
   - **h.** Review of systems/conclusion of the interview: Request the patient to think from head-to-toe, as well as through each organ system, to recall of any symptoms they missed for the final report.
5. Be aware of limitations
   Collecting information from the patient is a skillful activity. It is sometimes called an art. Some patients may be difficult to interview. Talkative, silent, depressed, confused, aggressive, or crying patients will need different approaches. For example, a talkative patient may need to be directed to focus and describe their problems, whereas a confused patient may need to be probed further. In addition, there will be culturally sensitive issues which need to be asked sensibly, such as dietary habits or alcohol use.[23] It is better to skip certain questions that complicate the situation; a noncooperating patient should not be asked questions that worsen the situation.
6. Answer the questions
   The patient's questions should be answered appropriately, as a diagnosis may not have been confirmed or misunderstood. Sometimes patients might gauge how the interviewer feels about the prognosis of their condition. Being genuine in addressing and acknowledging their problems and encouraging the patient to wait for a conclusion/clinical diagnosis, is a good strategy.
7. Develop a common empirical understanding of the condition
   Patients often seek a consultation to understand their condition. They need the expertise of a clinician to help them understand it. Development of a common understanding of the conditions will help both the patient and clinicians to plan an effective and feasible solution.
8. Prepare the patient for the next task and close the interview
   If necessary, the patient should be made aware why further examinations (e.g., lab tests) are required. Of course, they should be advised on how to solve their health problem, if a direct course of action is known. If the interview is not closed well, the patient may feel the session was performed incompletely. It will affect patient satisfaction during the care process.[18,19]

   Preferably the patient history interview should be performed exclusively. If not enough time is available, some clinicians may start with the interview and continue it while performing a physical assessment. However, such an approach may be only applicable to more experienced interviewers who are good at multitasking. Remember not to ask too many questions unless necessary. Short interventions should be linked to short history taking.

For those who like mnemonics, some of the classic mnemonics used in the patient history interview are as follows:

- Eight dimensions of a medical problem to collect the history of present illness; **OLDCARTS**—**O**nset, **L**ocation and/or radiation, **D**uration, **C**haracteristics (e.g., what does it feel like?), **A**ggravating and/or relieving factors, **R**elated symptoms, **T**reatments tried (including the patient's response), and **S**everity (usually on a scale of 0−10).
- Specifically for a patient complaining of pain, consider using **OPQRST**—**O**nset, **P**rovocation and palliation, **Q**uality of pain (e.g., what does it feel like?), **R**egion and radiation, **S**everity, and **T**iming (e.g., constant, intermittent, duration, time of day).
- **SOCRATES**—**S**ite, **O**nset, **C**haracteristics, **R**adiating, **A**lleviating, **T**iming, **E**xacerbating factors, and **S**everity.[18]

Some of these tools are linked to the provision of a service or are more specific for medication history taking. To make the best of these tools, a pharmacist should become familiar with their content and use them as a guide rather than as a formal, prescriptive method of history taking, as each patient is an individual with different needs.

Reflective Exercise

Prepare two checklists: (1) the steps of patient history interview (i.e., #1 through #8) and (2) OLDCARTS. Practice and perform a patient history interview with each method. Have your supervisor or peer check off each section as you progress systematically through the history-taking process. In addition, do not forget to tailor the session as a natural and convenient conversation for the patient.

CONCLUSION

Communication skills are essential for pharmacists to deliver their services to healthcare professionals and consumers successfully. It is a core competency which pharmacy students should develop through education. Communication skills are evident in the practice of professionals regarding their knowledge transfer, utilization, and behavior. Accreditation standards for healthcare facilities aim to decrease medication errors by effective communication. Appropriate healthcare practice involves teamwork and communication, which are essential to developing and implementing clinical decisions via consensus. Patient history interviews need high levels of communication skills to collect important information. Collecting vital information from all patients, including noncooperating ones, is essential in proper diagnosis and prognosis when objective evidence is limited.